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PRE- SUBMISSION PEER REVIEW

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Pre-Submission Review Sample

Reviewer-style rejection risk check report

Built from a real public manuscript with visible reporting and methodological weaknesses, then anonymized and reorganized into a deployment-ready sample.

Prepared for	ResearchEdit4u Solutions Private Limited
Use case	Website sample for Pre-Submission Review / Rejection Risk Check
Branding	RE4U branded sample document
Website	www.researchedit4u.in

Important note

This sample is intentionally simple to read, but it follows strong peer-review logic. It shows not only what is wrong section by section, but also what a careful human reviewer notices that a generic AI review often misses.

1. Executive Summary

Target verdict

Major revision required before submission. In its current form, this manuscript would be at high risk of rejection because the analytical claims are stronger than the design can support, the modeling strategy is unstable for the rare outcome group, and the reporting does not fully satisfy what editors and reviewers need to trust the findings.

Section scores

Domain	Score	Risk	Decision signal
Novelty and rationale	2.5	Moderate	Question is relevant but gap is not sharpened enough
Methods and design	1.5	High	Design and analytic choices weaken credibility
Statistical reporting	1.0	High	Sparse outcome group undermines model stability
Results presentation	2.0	Moderate	Findings are readable but over-interpreted
Discussion and limitations	1.5	High	Limitations are too narrow for the actual risks
Journal readiness	1.5	High	Likely reviewer pushback before external review

Fast rejection triggers

- The reference outcome group for multinomial logistic regression contains only 8 participants, which makes the adjusted odds ratios vulnerable to instability and inflated uncertainty.
- Cross-sectional data are repeatedly discussed using causal language such as predict, even though the design supports association, not direction or causation.
- The limitation section admits only sex imbalance, but does not adequately discuss nonresponse, selection bias, sparse cells, missing-data handling, or loss of information from collapsing variables.
- The model recoding strategy is presented after the table and some labels appear corrupted or inconsistent, which signals weak table QA and weak statistical supervision.

What a careful reviewer notices first

- Only 1.4 percent of the sample is in the unwilling group. That means the study is trying to explain hesitancy using a group too small to support a confident multivariable model.
- Several bivariate chi-square cells almost certainly contain very small expected counts because the unwilling group has only 8 cases spread across many categories.
- The paper says it follows STROBE, yet key STROBE-sensitive elements remain weak or absent, especially around bias, study size rationale, missing data, analytical handling of the sampling strategy, and sensitivity analysis.

- The narrative occasionally contradicts or softens its own limitations. The paper admits the chosen population may not be appropriate for hesitancy analysis, yet still offers practice-facing recommendations with high confidence.

2. Manuscript Basis Used for This Sample

For deployment, the report below refers to the source as an anonymized near-final cross-sectional manuscript. However, this sample was built from a real public preprint so that the reviewer logic is grounded in an authentic paper rather than an invented weak draft.

Source basis

Public source title	COVID-19 Vaccine Acceptance and Uptake Among Healthcare Workers in Trinidad & Tobago
DOI / preprint	medRxiv doi: 10.1101/2022.05.09.22274854
Study design	Cross-sectional survey with chi-square tests and multinomial logistic regression
Sample reported	N = 584; only 8 participants in the unwilling group
Why selected	Provides a realistic example of a manuscript that is topical and readable, yet still vulnerable to rejection because of reporting, modeling, and interpretation weaknesses.

Scope note

This report does not reproduce the paper extensively. It uses short factual references to the source sections, page lines, and tables, then converts those observations into a professional pre-submission review that your website visitors can easily understand.

3. Title, Abstract, and Framing

Score 2.0/5	Risk Moderate	Needs controlled revision before submission
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Why this section would trigger rejection

- The abstract states that age, profession, and trust predict vaccine uptake, but the study is cross-sectional. That wording overstates what the design can support.
- The abstract gives statistical methods but not enough caution about the very small unwilling group used as a reference category later in the model.
- The framing is relevant but not tight enough to define a precise gap beyond local descriptive relevance.

Hidden editor signals

- Editors often flag causal verbs in observational abstracts because they create downstream problems in peer review, media framing, and title fit.
- An abstract that sounds more certain than the methods section suggests poor internal calibration, which is a trust problem as much as a language problem.

Human-detail findings

- The paper moves from association language in some places to prediction language in others. That inconsistency is subtle but important because it reveals weak methodological discipline in the writing process.

- The conclusion in the abstract jumps directly to intervention-facing recommendations without first acknowledging the limited precision of the hesitancy subgroup.

Screen references to the source manuscript

- Abstract methods and results: p. 2, lines 30-49.
- Abstract conclusion: p. 3, lines 48-51.
- Aim statement: p. 5, lines 94-98.

Priority fixes before submission

- Replace predict with is associated with throughout the title, abstract, results discussion, and conclusion unless a predictive framework was actually developed and validated.
- Add one balancing sentence in the abstract noting that hesitancy was rare in the sample, which limits precision for multivariable comparisons.
- Tighten the gap statement so the manuscript explains what is unknown, not only what is important.

4. Introduction and Research Gap

Score 2.5/5	Risk Moderate	Needs controlled revision before submission
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Why this section would trigger rejection

- The introduction establishes topic importance, but the gap is broad and descriptive rather than sharply scientific.
- The rationale does not clearly explain why this specific regional sample adds enough incremental knowledge beyond earlier healthcare-worker vaccine acceptance studies.
- The introduction lists possible demographic and trust variables but does not convert them into a coherent conceptual model.

Hidden editor signals

- Editors ask a silent question: why this paper, in this journal, now? Here, the local context is present, but the incremental contribution remains under-specified.
- A literature review that catalogs mixed findings without synthesizing them into a testable tension often signals a weak discussion section later.

Human-detail findings

- The introduction cites contradictory evidence on sex and age, but does not define how the present design will resolve or meaningfully narrow that uncertainty.
- Trust is introduced as important, yet the manuscript does not clearly distinguish interpersonal trust, institutional trust, and media-related trust in a theory-led way.

Screen references to the source manuscript

- Background and rationale: p. 4, lines 76-98.
- Country context and local vaccination rollout: p. 5-6, lines 99-115.
- Local prior evidence on side effects: p. 6, lines 111-115.

Priority fixes before submission

- End the introduction with a tighter 3-part close: what is unknown, why this sample can help, and what exactly is being tested.
- Add a short conceptual frame for the trust variables so the reader knows why each trust domain belongs in the model.
- Clarify whether the manuscript aims to explain uptake, hesitancy, or both. At present it shifts between them.

5. Methods, Sampling, and Measurement

Score 1.5/5	Risk High	Likely reviewer resistance
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Why this section would trigger rejection

- Judgment sampling limits external validity and should be discussed as a design choice with direct implications for generalizability.
- The study pretested the questionnaire for clarity, but it does not report enough about measurement quality in the final sample, such as reliability or construct handling.
- The paper says it followed STROBE, yet methods reporting remains incomplete around bias, missing data, and study size rationale.

Hidden editor signals

- When a manuscript uses nonprobability sampling and then writes recommendations in a broad policy tone, editors worry the discussion will outrun the design.
- A reviewer often interprets limited instrument reporting as a sign that the variable definitions were treated as administratively convenient rather than scientifically robust.

Human-detail findings

- The sampling frame is restricted to selected worker categories who were likely involved in vaccine administration. That may systematically elevate uptake and reduce the very outcome the paper wants to model.
- The manuscript reports that 584 of 1,351 workers completed the questionnaire, but does not translate this into a formal response-rate discussion or nonresponse-bias reflection.
- There is no clear statement of how missing responses were handled. In survey studies, silence on missingness is not neutral.

Screen references to the source manuscript

- Judgment sampling and study participants: p. 7, lines 145-157.
- Instrument description and pretest: p. 8, lines 159-170.
- Ethics and analyses heading: p. 8-9, lines 176-193.
- STROBE claim: p. 7, lines 143-144.

Priority fixes before submission

- State the response rate explicitly and discuss how nonresponse may have biased both uptake and trust estimates.
- Report whether any item nonresponse occurred and how complete-case versus available-case analysis was handled.

- Describe how each trust variable was derived, whether categories were prespecified, and whether any reliability or internal-consistency assessment is available for grouped items.
- Add a frank sentence that the sampled roles may have been more vaccine-engaged than the wider healthcare workforce, which can distort hesitancy analysis.

6. Statistical Analysis and Model Validity

Score 1.0/5	Risk High	Likely reviewer resistance
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Why this section would trigger rejection

- The multinomial logistic regression uses the unwilling group as the reference category even though it contains only 8 participants. That is the single biggest rejection risk in the paper.
- The manuscript does not show whether expected cell counts were acceptable for the many chi-square tests carried out on a highly imbalanced outcome.
- Variable collapsing is done after the fact and presented briefly, with little justification for why information loss was acceptable.

Hidden editor signals

- Editors may not run the model themselves, but they recognize instability signals. Tiny reference groups, huge intervals, and many categorical predictors immediately reduce confidence.
- A messy or corrupted regression table label is often interpreted as a proxy for deeper analytical carelessness. Reviewers notice when a table looks under-checked.

Human-detail findings

- The paper reports 514 vaccinated, 62 unvaccinated but willing, and only 8 unwilling participants. A rare-category reference group can create quasi-separation and unstable adjusted odds ratios.
- Table 4 includes wide intervals and at least one obviously problematic confidence interval rendering, written as 1.23 (15-10.03), which suggests a table production or extraction error that should never survive into a near-final submission.
- The manuscript says trust and age were dichotomized. This may simplify the model, but it also discards ordinal information and can produce arbitrary thresholds if not prespecified and justified.
- No sensitivity analysis is reported. With such sparse outcome data, sensitivity checks are not optional if the authors want confidence in their conclusions.

Screen references to the source manuscript

- Analysis description: p. 8, lines 181-193.
- Outcome distribution: p. 10, lines 282-289.
- Chi-square interpretation and sex imbalance caveat: p. 10, lines 210-216.
- Collapsed-variable explanation for Table 4: p. 15, lines 255-263.
- Regression table and AORs: p. 16, lines 283-289.

Priority fixes before submission

- Rebuild the primary analysis. Consider a simpler outcome structure or a descriptive plus penalized or exact approach appropriate to rare events, depending on journal norms and data availability.

- Report expected-cell checks for chi-square tests, or replace weak comparisons with more suitable methods.
- Justify every recoding decision and show the original category structure in supplementary material.
- Add sensitivity analyses that test whether the main claims survive alternative coding or simpler models.
- Perform a full table QA pass so that all labels, confidence intervals, reference categories, and footnotes are statistically and typographically correct.

7. Results Presentation and Internal Consistency

Score 2.0/5	Risk Moderate	Needs controlled revision before submission
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Why this section would trigger rejection

- The results section is broadly readable, but the inferential narrative is too confident relative to the data structure.
- Several reported associations arise from extremely small unwilling-group counts distributed across many categories, which makes the descriptive percentages look more stable than they really are.
- The manuscript does not clearly separate what is descriptive, what is bivariate, and what remains credible after multivariable adjustment.

Hidden editor signals

- Editors dislike results sections that sound decisive while the tables imply fragility. That mismatch creates immediate reviewer skepticism.
- A results section that does not visually protect the reader from overinterpreting tiny subgroup numbers is often treated as a judgment problem, not only a formatting problem.

Human-detail findings

- The paper itself notes that the observed sex association in chi-square testing could be influenced by imbalance. That is a valuable admission, but it should have triggered a more cautious reporting tone across the whole section.
- The unwilling group is so small that percentages like 75.0 percent or 25.0 percent can be driven by only 6 versus 2 participants. A skilled reviewer mentally converts those percentages back to counts.

Screen references to the source manuscript

- Participant characteristics and outcome counts: p. 9-10, lines 204-209.
- Chi-square narrative for sex and age: p. 10, lines 210-218.
- Selected trust cross-tabs: p. 14, lines 471-489.
- Regression interpretation text: p. 15, lines 264-281.

Priority fixes before submission

- Rewrite the results so each statistical layer is clearly separated: descriptive profile, bivariate associations, then adjusted model findings with explicit caution.

- Where a subgroup is extremely small, report the raw count before the percentage in the narrative.
- Use language such as suggestive, unstable, or should be interpreted cautiously where the data structure demands it.

8. Discussion, Limitations, and Conclusion

Score 1.5/5	Risk High	Likely reviewer resistance
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Why this section would trigger rejection

- The discussion compares findings with prior studies, but it does not spend enough time on why the current design may be unable to support strong claims about hesitancy drivers.
- The limitation section is too narrow. It mentions sex imbalance and hints that the population may be poor for hesitancy evaluation, but it does not fully unpack the consequences.
- The conclusion still delivers action-oriented recommendations despite acknowledged design weakness.

Hidden editor signals

- When authors recognize a core design problem but then keep confident policy recommendations, editors read that as weak inferential discipline.
- A conclusion that sounds cleaner than the methods deserve often pushes an editor toward reject and resubmit rather than revise and review.

Human-detail findings

- The manuscript states that the chosen population may not have been appropriate for evaluating factors associated with hesitancy. That is a major admission and should have materially softened the claims across the abstract, discussion, and conclusion.
- The limitations do not address common-method bias from self-report, the selected worker roles, the possibility of socially desirable responding in a professional population, or uncertainty created by rare-event modeling.

Screen references to the source manuscript

- Discussion opening: p. 16-18, lines 290-345.
- Trust discussion: p. 19, lines 346-353.
- Limitation statement: p. 19, lines 354-359.
- Conclusion statement: p. 19, lines 360-367.

Priority fixes before submission

- Expand limitations into a real methodological accounting, not a brief compliance paragraph.
- Pull back from intervention-style recommendations unless they are explicitly framed as tentative and hypothesis-generating.
- Align the conclusion with what the data can truly support: local associations in a selected sample, not firm determinants of hesitancy.

9. Tables, Reporting Quality, and Journal Readiness

Score 1.5/5	Risk High	Likely reviewer resistance
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Why this section would trigger rejection

- Table 4 is not publication-ready. It contains label problems, compressed logic, and at least one confidence interval entry that appears malformed.
- The manuscript says it follows STROBE but does not clearly show how key reporting items were satisfied.
- Overall presentation is readable, but not yet polished enough for a near-final submission to a quality journal.

Hidden editor signals

- Editors are heavily influenced by tables because tables expose whether the underlying analysis was controlled, checked, and honestly reported.
- A single visibly broken table can reduce trust in the full manuscript, even if the rest of the paper seems fluent.

Human-detail findings

- The line Trust in: International television broadcasts appears in the regression table region, even though the main trust domain appears to be international organizations. Whether this is extraction noise or manuscript noise, the submitted table must be cleaned before peer review.
- The note that omitted parameters were set to zero because of redundancy is technically plausible, but the table does not explain the coding structure clearly enough for an independent reader to reconstruct the model.

Screen references to the source manuscript

- Regression table labels and note: p. 16, lines 283-289.
- STROBE statement: p. 7, lines 143-144.
- Data availability statement: p. 20, lines 385-387.

Priority fixes before submission

- Redesign Table 4 from scratch with cleaner labeling, explicit reference categories, clearly grouped predictors, and corrected confidence intervals.
- Add a reporting-checklist appendix for cross-sectional studies before submission.
- Run a final editorial QA pass focused only on tables, footnotes, cross-references, and consistency of terminology.

10. Final Reviewer Recommendation

Decision

Reject in current form for direct submission. Encourage substantial methodological and reporting revision before the manuscript is sent to a journal.

Why we would reject as a reviewer

- The central inferential model is built on a reference group that is too small to carry the weight of the conclusions.
- The discussion and conclusion remain more confident than the data structure allows.
- The manuscript does not yet demonstrate the level of statistical and reporting control expected for a near-final submission.

Why we would reject as an editor

- Even before external review, the regression strategy raises a reliability concern that could lead to avoidable reviewer criticism.
- The paper would likely consume reviewer time on problems that should have been fixed before submission.
- The manuscript is not fatally irrelevant, but it is not yet journal-ready.

Submission rescue plan

Step	Action	What to fix	Impact
1	Reframe claims	Replace causal or predictive wording with association language throughout.	High
2	Rework analysis	Use a method appropriate for rare hesitancy outcomes and justify all coding decisions.	High
3	Deepen limitations	Add nonresponse, selection bias, sparse data, self-report bias, and generalizability limits.	High
4	Repair tables	Rebuild regression table and cross-check all labels, intervals, and notes.	High
5	Align narrative	Ensure abstract, results, discussion, and conclusion carry the same level of caution.	High

11. Best-Practice Standards Used to Shape This Review

This sample follows a constructive peer-review approach. It is not written to embarrass an author. It is written to show what a strong pre-submission review should catch before journal submission.

- COPE guidance emphasizes that peer review should be objective, constructive, and focused on the quality and rigour of the work.
- ICMJE recommendations emphasize clear, accurate, reproducible reporting and appropriate use of reporting guidelines.
- STROBE expectations for cross-sectional studies highlight transparent reporting of bias, study size, statistical methods, handling of missing data, participant flow, and cautious interpretation.

Source references used for building this sample

- Gopaul CD, Ventour D, Thomas D. COVID-19 Vaccine Acceptance and Uptake Among Healthcare Workers in Trinidad & Tobago. medRxiv. doi:10.1101/2022.05.09.22274854.
- COPE. Ethical guidelines for peer reviewers.
- ICMJE. Recommendations for the Conduct, Reporting, Editing, and Publication of Scholarly Work in Medical Journals.
- STROBE Statement checklist for cross-sectional studies.

End of sample

THANK YOU

PRE-SUBMISSION PEER REVIEW TOOLKIT SAMPLE

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